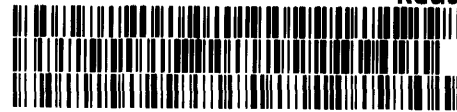


UID:DD442847-AE68-4492-AE4A-0BCD1AC15A0C
TCGA-XF-A9ST-01A-PR

Redacted



Collected:
Ordered by:

FINAL DIAGNOSIS:

RADICAL CYSTOPROSTATECTOMY WITH BILATERAL PELVIC LYMPH NODE DISSECTION:

RIGHT URETER (A):

FROZEN SECTION DIAGNOSIS:

- NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER

- NO TUMOR IDENTIFIED

LEFT URETER (B):

FROZEN SECTION DIAGNOSIS:

- HYDROURETER, NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- HYDROURETER

- NO TUMOR IDENTIFIED

BLADDER AND PROSTATE (C):

GROSS DIAGNOSIS:

- MARGINS GROSSLY FREE OF TUMOR

BLADDER:

- INVASIVE PAPILLARY TRANSITIONAL CELL CARCINOMA, HIGH GRADE (3-4/4), MEASURING 10 CM IN GREATEST DIMENSION, WITH EXTENSIVE NECROSIS
- TUMOR EXTENDS INTO PERIVESICAL SOFT TISSUE
- LYMPHOVASCULAR INVASION IDENTIFIED
- RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY (URETER MARGINS, RADIAL AND APICAL MARGINS OF SPECIMEN)
- MULTIFOCAL UROTHELIAL CARCINOMA IN SITU (FLAT LESION)
- RANDOM BLADDER MUCOSA SHOWING REACTIVE UROTHELIAL ATYPYA AND ACUTELY AND CHRONICALLY INFLAMED DIVERTICULUM
- LEFT URETER SHOWING HYDROURETER, MILD

PROSTATE:

- BENIGN PROSTATIC GLANDULAR AND STROMAL HYPERPLASIA
- NO HIGH GRADE INTRAEPITHELIAL NEOPLASIA OR MALIGNANCY IDENTIFIED
- DISTAL PROSTATE, APICAL AND RADIAL MARGINS, FREE OF TUMOR

RIGHT AND LEFT PERIVESICAL LYMPH NODES:

- NO TUMOR IDENTIFIED IN ONE LYMPH NODE (0/1)
- EXTENSIVE VASCULAR CALCIFICATIONS IDENTIFIED

RIGHT PELVIC LYMPH NODES (D):

- NO TUMOR IDENTIFIED IN THREE LYMPH NODES (0/3)

LEFT PELVIC LYMPH NODES (E):

- NO TUMOR IDENTIFIED IN NINE LYMPH NODES (0/9)

LEFT URETERAL MARGIN (F):

- NO TUMOR IDENTIFIED

RIGHT PROXIMAL URETER (G):

- NO TUMOR IDENTIFIED

LYMPH NODE SUMMARY: NO MALIGNANCY IDENTIFIED IN 13 LYMPH NODES (0/13)

PATHOLOGIC TNM STAGE: BLADDER: pT3bN0MX

path *ICD0-3*
Carcinoma, urothelial papillary
81301/3
Carcinoma, transitional cell papillary
81301/3
Site *Bladder NOS* *C67.9*
path *Trigone & bladder* *C67.0*
7/26/14

collected
Ordered by:

DIAGNOSIS COMMENT:

Representative tumor tissue is submitted for p53 assay by immunohistology, results of which will be issued in an addendum.

INTRAOPERATIVE DIAGNOSIS:

FROZEN SECTION DIAGNOSIS:

AFS: Right ureter:

- no tumor identified

BFS: Left ureter:

- hydroureter, no tumor identified

GROSS DIAGNOSIS:

Bladder/prostate:

- margins grossly free of tumor

GROSS DESCRIPTION:

A: The specimen is received fresh from the O.R. and labeled "Right ureter." It consists of a segment of ureter that measures 1 x 0.5 x 0.3 cm. The specimen is totally submitted for frozen section in cassette AFS.

B: The specimen is received fresh from the O.R. and labeled "Left ureter." It consists of a segment of ureter that measures 0.8 x 0.5 x 0.3 cm. The specimen is totally submitted for frozen section in cassette BFS.

C: The specimen is received fresh from the O.R. and labeled "Bladder/prostate." It consists of a cystoprostatectomy specimen measuring overall 25 x 16 x 6 cm. The attached peritoneal fat measures 20 x 15 x 0.1 cm and is smooth and glistening. The bladder itself measures 9 x 10 x 5 cm. The resection margins are inked black. The bladder is opened along the anterior prostatic urethra and bladder wall to show the bladder to be filled with clotted blood measuring 8 x 5 x 3 cm. In addition, there is a fungating tumor mass measuring 10 x 8 x 3 cm at the level of the left trigone extending into the midline and disrupting the ureterovesical junction. A second tan-pink flat granular mass is located in the bladder neck measuring 1.5 x 1 x 1 cm. A diverticulum is located adjacent to the primary tumor mass measuring 2.5 x 3 cm. Opening the ureters bilaterally shows the right distal ureter to measure 8 cm in length and 0.8 cm in circumference. The left distal ureter measures 4 cm in length and 1.5 cm in circumference, shows mild hydroureter, and is surrounded by tumor that does not extend into the mucosa. Serial sectioning through the tumor shows an underlying submucosal globular structure measuring 1.5 x 1.5 cm. In addition, the tumor extends grossly to the muscularis propria to involve the surrounding perivesical fat. Sections through the second mass shows it to appear to be limited to the mucosal surface. The prostate measures 5 x 3 x 3 cm. The prostatic urethra is 4 cm long and 3.5 cm in circumference. The verumontanum is 1 x 0.4 x 0.4 cm. The left seminal vesicle is 4 x 1.5 x 1 cm and the right seminal vesicle is 4 x 2 x 1 cm. Sections through the prostate reveals a tan, firm nodular cut surface. No discrete lesions are identified grossly. Representative sections are submitted in 34 cassettes.

D: The specimen is received in formalin and labeled "Right pelvic lymph nodes." It consists of multiple fragments of fat measuring 8 x 4 x 1 cm. Two lymph nodes are identified within the fatty tissue measuring 7 x 2 x 0.8 cm and 1.5 x 1 x 0.4 cm. The largest lymph node is quadrisected and placed in cassettes D1 through D3, the second lymph node is also placed in cassette D3, and remaining tissue is placed in cassette D4.

Collector:
Ordered by:

E: The specimen is received in formalin and labeled "Lt pelvic LN." It consists of multiple fragments of fatty tissue measuring 7.8 x 5 x 1.2 cm in aggregate. Seven possible lymph nodes are identified ranging from 0.2 to 3.5 cm in greatest dimension. The largest lymph node is bisected and placed in cassette E1 and remaining tissue is placed in cassettes E2 through E4, four cassettes total.

F: The specimen is received in formalin and labeled "Left ureteral margin." It consists of a tan tubular segment of tissue with attached fat measuring 6.5 x 2.5 x 0.4 cm. The ureter itself measures 1.5 cm in length and 0.4 cm in diameter. A clip and black suture is located off center, therefore, the end closest to the clip is inked black and the other end is inked red. The specimen is serially sectioned and entirely submitted in two cassettes.

G: The specimen is received in formalin and labeled "Rt proximal ureter." It consists of a segment of tan tubular tissue with attached fat with two clips placed equidistance from either end measuring 1 cm in length and 0.3 cm in diameter. Both ends are inked black. The specimen is serially sectioned and entirely submitted in one cassette.

SECTIONS:

AFS: frozen section, right ureter
BFS: frozen section, left ureter
C1: apex margin of specimen
C2: right ureter
C3: left ureter
C4: tumor left lateral
C5: tumor left lateral
C6: tumor and diverticulum
C7: diverticulum
C8,9: full thickness of tumor to fat
C10,11: full thickness of tumor to nodule
C12: tumor bladder neck
C13: tumor to right ureterovesical junction
C14: tumor to left ureterovesical junction
C15: right posterior bladder wall
C16: dome of bladder
C17: left posterior bladder wall
C18: right anterior bladder wall
C19: left anterior bladder wall
C20: right distal prostate
C21: left distal prostate
C22: right anterior midprostate
C23: right posterior midprostate
C24: left anterior midprostate
C25: left posterior midprostate
C26: right anterior proximal prostate
C27: right posterior proximal prostate to seminal-vesical junction
C28: left anterior proximal prostate
C29: left posterior proximal prostate to seminal-vesical junction
C30-34: lymph nodes
D1-3: right pelvic lymph nodes; largest lymph node
D4: remaining tissue
E1: left pelvic lymph node; largest lymph node
E2-4: remaining tissue
F1,2: left ureteral margin all embedded
G: right proximal ureter all embedded

MICROSCOPIC DESCRIPTION:

A-B: See final microscopic-diagnosis.

Collected:

Ordered by:

Location:

C: Sections of the bladder show high-grade invasive papillary transitional cell carcinoma, grade 3-4/4 with extensive necrosis, extending through the muscularis propria to involve the perivesical soft tissue (C5,11,14). Carcinoma in situ is also identified (C12). Lymphovascular invasion is identified (C6). The bilateral ureter margins of the specimen, radial and apical margins are negative for tumor. The uninvolved bladder shows reactive urothelial atypia as well as an acutely and chronically inflamed diverticulum. The left ureter shows mild hydroureter (C3).

Sections of the prostate show benign prostatic glandular and stromal hyperplasia without dysplasia or malignancy. The distal prostate, apical and radial inked margins are negative for tumor. Extensive vascular calcifications are identified within the perivesical fat. One lymph node is identified and is free of tumor.

D-G: See final microscopic-diagnosis.

Signed by:

ELECTRONIC SIGNATURE

PRE-OPERATIVE DIAGNOSIS:

Bladder CA

ORDERING PHYSICIAN:

Ordering Physician:

Criteria	lw 1/11/14	Yes	No
Diagnosis Discrepancy			/
Primary Tumor Site Discrepancy			/
HIPAA Discrepancy			/
Prior Malignancy History			/
Dual/Synchronous Primary Noted			/
Case is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer Initial:	JAC	Date Reviewed:	1/3/2014